



R3 Recovery Services

618 Columbia Ave, Lancaster, [REDACTED] US, 17603
(717) 349-9924

Date: 06/11/2026

CLIENT PROFILE



Code Status: NA
SSN: [REDACTED]
MRN Number: [REDACTED]
Client Id: NA
Gender: Male
Pronoun(s): He/him/his
Birth Date: [REDACTED]
Phone Number: [REDACTED]

Primary Clinician: Baker Hodges
Status: Active
Admission Date: 02/25/2026
Discharge Date: 02/25/2026
Medical Provider:
Level Of Care: IOP 5
Treatment:

Address: [REDACTED] [REDACTED] [REDACTED] United States,19608
Religion: no religious affiliation

Reviewer [REDACTED]

Marleigh Johnson, MS, LPC
February 25, 2026 03:06 PMTreatment Plan: **02/26/2026 -
02/26/2027**Primary Clinician: **Saffron Brocious**Expected Length of Treatment
(days): **365****Problem:** Substance Use Disorders**Diagnostic Impression:****02/26/2026** - [F10.20] Alcohol use disorder, Severe**02/26/2026** - [F14.20] Cocaine use disorder, Severe**Behavioral Definitions:****02/26/2026** - Has a history of arrests for addiction-related offenses (e.g., driving under the influence [DUI], minor in possession [MIP], assault, possession/delivery of a controlled substance, shoplifting, breaking and entering [B&E]).**02/26/2026** - Demonstrates a maladaptive pattern of substance use, manifested by increased tolerance and withdrawal.**02/26/2026** - Fails to stop or cut down use of mood-altering drug once started, despite the verbalized desire to do so and the negative consequences continued use brings.**02/26/2026** - Denies that chemical dependence is a problem, despite feedback from significant others that the use of the substance is negatively affecting him or her and others.**Client Goal:** Utilize behavioral and cognitive coping skills to help maintain sobriety.**Objective:** [REDACTED] will develop and implement effective coping skills to manage symptoms of depression and anxiety, leading to a reduction in overall distress and improvement in daily functioning.**Intervention:** Teach him mindfulness meditation, deep breathing exercises, or progressive muscle relaxation to help manage acute anxiety.**Intervention:** Work with [REDACTED] to identify negative thought patterns and replace them with more positive or realistic thoughts.**04/27/2026: Status** - (Open)**Objective:** [REDACTED] will cooperate with exploration of increasing satisfaction in areas of life that can support sobriety such as employment, recreation, and relationships.**Intervention:** Work with [REDACTED] to find the path he wants to pursue in school and his career. Encourage him to build relationships and connections.**04/27/2026: Status** - (Open)**Client Goal:** Establish a sustained recovery, free from the use of all mood-altering substances.**Objective:** [REDACTED] will commit self to an action plan directed toward termination of substance use.**Intervention:** Encourage and support the client's self-efficacy for change toward the goal of developing an action plan for termination of substance use to which the client is willing to commit.**04/27/2026: Status** - (Open)**Objective:** [REDACTED] will learn and implement relapse prevention strategies for managing possible future situations with high risk for relapse.**Intervention:** Use stimulus control techniques such as avoidance of specific triggers to reduce exposure to high-risk situations.**04/27/2026: Status** - (Open)**Objective:** [REDACTED] will attend group therapy sessions where he will process through recovery and mental health related issues in a supportive environment.**Intervention:** Encourage [REDACTED] to attend group therapy sessions and be open about his experiences with substance use.**04/27/2026: Status** - (Open)

Treatment Plan Addendum

Level of Care:

- ☐ PHP (2.5)
- ☒ IOP 19 (2.1)
- ☐ IOP 5 (2.1)
- ☐ IOP 3 (2.1)
- ☐ OP (1.0)

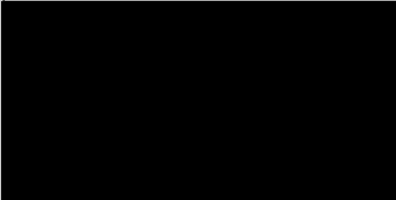
Treatment Interventions to Support Established Treatment Plan:

- ☒ Individual Sessions
 - ☒ 1x per week
 - ☐ 2x per week
 - ☐ Other:
- ☒ Group Sessions
 - ☒ 5x per week
 - ☐ 4x per week
 - ☐ 3x per week
 - ☐ 2x per week
 - ☐ 1x per week
 - ☐ Other:



February 26, 2026 02:36 PM

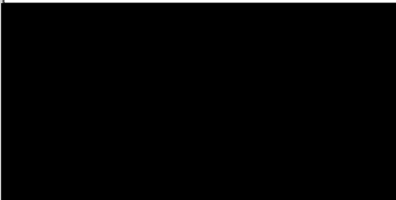
Staff



Saffron Brocious, BA

February 26, 2026 02:36 PM

Reviewer



Marleigh Johnson, MS, LPC

February 26, 2026 02:36 PM

Treatment Plan: 02/26/2026 - 02/26/2027	Primary Clinician: Baker Hodges	Expected Length of Treatment (days): 365
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Intervention: Instruct the client to routinely use strategies learned in therapy (e.g., problem-solving, stimulus control, social skills, and assertiveness) while managing high-risk trigger situations (or assign "Aftercare Plan Components" in the Adult Psychotherapy Homework Planner by Jongsma).

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[REDACTED]	
[REDACTED]	[REDACTED]
April 22, 2026 01:40 PM	February 26, 2026 02:36 PM

Staff [REDACTED]	
[REDACTED]	[REDACTED]
Baker Hodges, MS April 22, 2026 01:40 PM	Saffron Brocious, BA February 26, 2026 02:36 PM

Reviewer [REDACTED]	
[REDACTED]	
Marleigh Johnson, MS, LPC February 26, 2026 02:36 PM	

----- END OF TX PLAN -----

04/19/2026

NA

Type (Check One):

☒ Admission☐ Continued Stay☐ DischargeDimension

	Level of Risk	Level of Care	Criteria Included/Comments
D1. Acute intoxication and/or withdrawal potential:	1	2.1	██████ reports last alcohol use on February 21, 2026, consuming approximately 11 mixed drinks in one sitting, with a pattern of drinking 5 to 6 days per week prior to admission. Cocaine use historically occurred in conjunction with alcohol, with reported use of 1 to 2 grams every other day at peak. He denies current withdrawal symptoms at intake and presents medically stable. ██████'s cravings have lessened in intensity, indicating progress in this area. Given his history of polysubstance use, there is still some risk, but ██████ has maintained sobriety since beginning treatment.
D2. Biomedical Conditions and Complications:	1	2.1	██████ denies chronic medical conditions, major injuries, communicable diseases, or life threatening allergies. He is not currently under active medical treatment aside from routine care and reports last physical approximately one year ago. No biomedical complications are currently interfering with participation in treatment
D3. Emotional/Behavioral or cognitive conditions and complications:	2	2.1	██████ carries diagnoses of Major Depressive Disorder, Generalized Anxiety Disorder, and Post Traumatic Stress Disorder. He has a significant trauma history involving chronic childhood physical and verbal abuse by his father. He reports multiple past suicide attempts beginning at age 13, including hanging attempts, self harm, reckless behaviors, and most recently a suicide attempt involving jumping from a cliff, which required hospitalization. ██████'s GAD 7 and PHQ 9 scores have gone down to 8 and 15 respectively, indicating some improvement on his anxiety and depressive symptoms. However, with ██████'s history of symptoms and still relatively high scores, he remains at moderate risk and should improve with further intensive outpatient treatment.
D4. Readiness to change:	2	2.1	██████ demonstrates insight into his substance use disorder and acknowledges loss of control, legal consequences, and mental health deterioration related to use. He expresses desire to be sober, develop coping skills, and find purpose. ██████ has become more open to sharing in group, stating that he is getting out of it more now than he was before, but that sometimes he just doesn't feel like sharing. Some of that barrier has come from being in group sessions with people he does not like, as he states he does not want some people knowing that much about him. Another barrier has been feeling like a burden, that he is just unloading all of his problems and no one wants to hear them. IOP will support continued motivation enhancement and accountability.

	Level of Risk	Level of Care	Criteria Included/Comments
D5. Relapse, continued use or continued problem potential:	2	2.1	████ has a severe alcohol and cocaine use disorder with early onset substance exposure and strong family history of addiction. He relapsed during prior IOP following residential and PHP treatment, particularly when structure decreased and he reconnected with using peers. He reports strong and frequent cravings, with boredom identified as a primary trigger. He also demonstrates cross addictive behaviors, including compulsive sexual behaviors during early recovery. Given recent relapse, ongoing cravings, legal stressors, trauma triggers, and history of heavy polysubstance use, relapse potential is high. █████ has made progress on several of these behaviors during his time in the program, but still maintains a moderate level of risk as more stressors become introduced to his life.
D6. Recovery Environment:	2	2.1	████ recently transitioned into R3 House, which provides increased structure and accountability, representing a supportive recovery environment. He reports strong support from his mother, stepfather, and uncle. However, he also maintains access to prior peer groups associated with substance use and has a rental property connected to a friend. Family history is heavily impacted by addiction, limiting long term sober modeling. Legal charges remain pending, creating environmental stress. While █████ has stated some benefits of living at the R3 House, he has also become somewhat tired of his roommates, and expressed that he would like to move out sooner rather than later. █████'s BARC-10 score also decreased since he entered treatment, which he has attributed to stress at the R3 House. █████ has been used to having his own space, and being around so many other people that are also struggling has been difficult for him.

Indicate level of care recommended:

2.1

Indicate level of care received:

2.1

If recommended level of care is different from received, why?

N/A

Indicate the program or Facility referred to:

R3 Recovery Services

Brief Assessment of Recovery Capital (BARC-10)

Brief Assessment of Recovery Capital (BARC-10)

Instructions: On a scale of 1 (Strongly disagree) to 6 (Strongly agree), please indicate your level of agreement with the following statements.

1. There are important things to me in life than using substances.

████ 1 Strongly Disagree █████ 2 Disagree █████ Somewhat Disagree █████ 4 Somewhat Agree █████ 5 Agree █████ 6 Strongly Agree

2. In general I am happy with my life.

████ 1 Strongly Disagree █████ 2 Disagree █████ Somewhat Disagree █████ 4 Somewhat Agree █████ 5 Agree █████ 6 Strongly Agree

3. I have enough energy to complete the tasks I set myself.

████ 1 Strongly Disagree █████ 2 Disagree █████ 3 Somewhat Disagree █████ 4 Somewhat Agree █████ 5 Agree █████ 6 Strongly Agree

4. I am proud of the community I live in and feel part of it.

☐ 1 Strongly Disagree	☐ 2 Disagree	☐ 3 Somewhat Disagree	☐ 4 Somewhat Agree	☐ 5 Agree	☐ 6 Strongly Agree
5. I get lots of support from friends.					
☐ 1 Strongly Disagree	☐ 2 Disagree	☐ 3 Somewhat Disagree	☐ 4 Somewhat Agree	☐ 5 Agree	☐ 6 Strongly Agree
6. I regard my life as challenging and fulfilling without the need for using drugs or alcohol.					
☐ 1 Strongly Disagree	☐ 2 Disagree	☐ 3 Somewhat Disagree	☐ 4 Somewhat Agree	☐ 5 Agree	☐ 6 Strongly Agree
7. My living space has helped to drive my recovery journey.					
☐ 1 Strongly Disagree	☐ 2 Disagree	☐ 3 Somewhat Disagree	☐ 4 Somewhat Agree	☐ 5 Agree	☐ 6 Strongly Agree
8. I take full responsibility for my actions.					
☐ 1 Strongly Disagree	☐ 2 Disagree	☐ 3 Somewhat Disagree	☐ 4 Somewhat Agree	☐ 5 Agree	☐ 6 Strongly Agree
9. I am happy dealing with a range of professional people.					
☐ 1 Strongly Disagree	☐ 2 Disagree	☐ 3 Somewhat Disagree	☐ 4 Somewhat Agree	☐ 5 Agree	☐ 6 Strongly Agree
10. I am making good progress on my recovery journey.					
☐ 1 Strongly Disagree	☐ 2 Disagree	☐ 3 Somewhat Disagree	☐ 4 Somewhat Agree	☐ 5 Agree	☐ 6 Strongly Agree

Scoring

Total scores can range from a minimum of 10 to a maximum of 60. A ROC Curve Analysis showed the BARC-10 had predictive validity with sustained remission (i.e., 1 year or more) using a cut-off score of 47 in a sample whose average length of recovery time was 7 years.

38

Generalized Anxiety Disorder 7-item (GAD-7) Scale**GAD-7**

Over the last 2 weeks, how often have you been bothered by the following problems?

1. Feeling nervous, anxious or on edge	☐ Not at all	☐ Several days	☐ More than half the days	☐ Nearly every day
2. Not being able to stop or control worrying	☐ Not at all	☐ Several days	☐ More than half the days	☐ Nearly every day
3. Worrying too much about different things	☐ Not at all	☐ Several days	☐ More than half the days	☐ Nearly every day
4. Trouble relaxing	☐ Not at all	☐ Several days	☐ More than half the days	☐ Nearly every day
5. Being so restless that it is hard to sit still	☐ Not at all	☐ Several days	☐ More than half the days	☐ Nearly every day
6. Becoming easily annoyed or irritable	☐ Not at all	☐ Several days	☐ More than half the days	☐ Nearly every day
7. Feeling afraid as if something awful might happen	☐ Not at all	☐ Several days	☐ More than half the days	☐ Nearly every day

GAD7 Score:

8

If you checked off any problems, how difficult have these problems made it for you to do your work, take care of things at home, or get along with other people?

☐ Not difficult at all	☐ Somewhat difficult	☐ Very difficult	☐ Extremely difficult
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Patient Health Questionnaire (PHQ-9)

PATIENT HEALTH QUESTIONNAIRE (PHQ-9)

Over the last 2 weeks, how often have you been bothered by any of the following problems?

1. Little interest or pleasure in doing things.

☐ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

2. Feeling down, depressed, or hopeless.

☐ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

3. Trouble falling asleep, staying asleep, or sleeping too much.

☐ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

4. Feeling tired or having little energy.

☐ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

5. Poor appetite or overeating.

☐ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

6. Feeling bad about yourself - or that you are a failure or have let yourself or your family down

☐ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

7. Trouble concentrating on things such as ☐ the newspaper or watching television.

☐ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

8. Moving or speaking so slowly that other people could have noticed? Or the opposite - being so fidgety or restless that you have been moving around a lot more than usual

☐ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

9. Thoughts you would be better off dead or of hurting yourself in some way

☐ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

PHQ-9 Score:

15

10. If you checked off any problems, how difficult have these problems made it for you to do your work, take care of things at home, or get along with other people?

☐ Not difficult at all ☐ Somewhat difficult ☐ Very difficult ☐ Extremely difficult

Columbia Suicide Risk Assessment

Columbia Suicide Risk Assessment

Ask questions 1 and 2. If both are negative, proceed to 6 "Suicidal Behavior" section. If the answer to question 2 is "yes", ask questions 3,4, and 5.

1. Wish to be Dead

- Client endorses thoughts about a wish to be dead or not alive anymore or wish to fall asleep and not wake up.

Have you wish you were dead or wished you could go to sleep and not wake up since last contact?

☐ Yes ☐ No

2. Non-Specific Active Suicidal Thoughts

- General, non-specific thoughts of wanting to end one's life/commit suicide without thoughts of ways to kill oneself/associated methods, intent, or plan during the assessment period.

Have you actually had any thoughts of killing yourself since last contact?

☐ Yes ☐ No

3. Active Suicidal Ideation with Any Methods (Not Plan) without Intent to Act

- Client endorses thoughts of suicide and has thought of at least one method during the assessment period. This is different than a specific plan with time, place, or method details worked out

Have you been thinking about how you might do this?

☐ Yes ☐ No

4. Active Suicidal Ideation with Some Intent to Act, without Specific Plan

- Active suicidal thoughts of killing oneself and subject reports having some intent to act on such thoughts.

Have you had these thoughts and had some intention of acting on them?

☐ Yes ☐ No

5. Active Suicidal Ideation with Specific Plan and Intent

- Thoughts of killing oneself with details of plan fully or partially worked out and subject has some intent to carry it out.

Have you started to work out or worked out the details of how to kill yourself? Do you intend to carry out this plan?

☐ Yes ☐ No

6. Suicidal Behavior

- A potentially self-injurious act committed with at least some wish to die, as a result of the act. Intent does not have to be 100%. If there is any intent/desire to die associated with the act, then it can be considered an actual suicide attempt. There does not have to be any injury or harm, just the potential for injury or harm. Also, if someone denies intent to die, but they thought that what they did could be lethal, intent may be inferred.

Have you done anything started to do anything, or prepared to do anything to end your life since last contact?

☐ Yes ☐ No

Summary and Recommendations

1. Overall Risk Assessment:

2. Additional Recommendations:

Treatment Plan LOC addendum

Level of Care

☐ PHP ☐ IOP 5 ☐ IOP 3 ☐ OP ☐ Other:

Group sessions

☐ 5 Groups per week ☐ 4 Groups per week ☐ 3 Groups per week ☐ 2 Groups per week ☐ 1 Group per week ☐ Other:

Individual sessions

☐ 1 session per week ☐ 2 sessions per week ☐ Other:

Apr 22 2026 1:40PM

Staff ☐

Baker Hodges, MS
Apr 22 2026 1:40PM

Reviewers

Marleigh Johnson, MS, LPC
Apr 27 2026 6:58AM

Treatment Plan: 02/26/2026 - 02/26/2027

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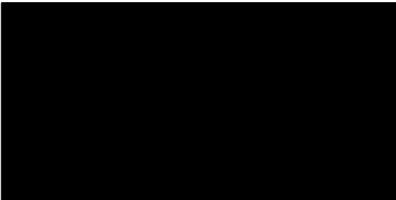
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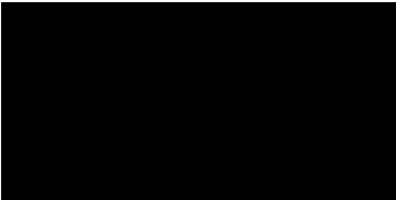
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----- END OF TX PLAN REVIEW -----

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DOB : [REDACTED]

MRN : [REDACTED]